

Risk Summary

Preeclampsia

Medium Score: 0.6395082154771051

Model Interpretation: Risk-based triage only (not diagnosis).

Macrosomia

Medium Score: 0.5943816940585895

Model Interpretation: Risk-based coaching support.

Summary: Patient presents at 32 weeks gestation with medium risk for preeclampsia, indicated by elevated systolic blood pressure and positive proteinuria. There is also a medium risk for macrosomia, influenced by high pre-pregnancy BMI and borderline fasting glucose, despite current GDM diagnosis status.

Patient Snapshot (Recent Inputs)

Parameter	Value	Notes
Blood Pressure	NA/NA mmHg	Hypertensive threshold ≥140/90, severe ≥160/110.
Proteinuria (dipstick)	NA	≥1+ supports PE suspicion in context of HTN.
Symptoms	Headache=NA, Vision=NA, RUQ Pain=NA, Swelling=NA	Severe features increase escalation priority.
Glucose	Fasting=NA mg/dL 1hr PP=NA mg/dL	Targets typically fasting <95, 1hr <140 (pregnancy goals).
HbA1c	NA	Higher HbA1c indicates sustained glycemic elevation.
Maternal Context	Age=NA BMI=NA GDM=NA	Risk modifiers for fetal overgrowth + metabolic risk.
Activity / Sleep	Post-meal walk=NA min Sleep=NA hrs	Lifestyle factors influence glycemic response.
Food Log		Used for dietary correlation patterns.

Reason for Alert

Action Requested: ALERT_DOCTOR

Clinical Rationale: Clinical Assistant Alert: Patient TEST_MED_MED (32 weeks gestation) has a medium risk for Preeclampsia and a medium risk for Macrosomia. For Preeclampsia, key findings include systolic BP 142 mmHg, diastolic BP 79 mmHg, and proteinuria dipstick 1. No severe symptoms reported. For Macrosomia, contributing factors are pre-pregnancy BMI 42.4 kg/m², maternal age 38 years, and fasting glucose 99 mg/dL (GDM not diagnosed). HbA1c is 5.6% and postprandial glucose is 126 mg/dL. Recommend clinical evaluation within 24 hours for preeclampsia risk and ongoing GDM/macrosomia management review.

Suggested Next Steps

- Review trend: BP, symptoms, proteinuria, glucose readings
- Consider confirmatory tests (protein/creatinine ratio, LFTs, platelets) if clinically indicated
- Consider GDM diet review or insulin adjustment if glucose targets exceeded
- Provide patient education + return precautions

⚠ This report is generated by an AI decision-support system and does not replace clinical judgment. Always follow institutional protocols.